

THE

Geriatrician's Guide

A FREE CHAPTER FROM THE LONG ROAD

What Senior Care Actually Costs

Nationally in 2026, assisted living runs around \$6,200 a month on average. But the average hides an enormous spread – and the advertised price is almost never what you'll actually pay.



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No placement commissions.
No facility money. Ever.

What Senior Care Actually Costs

What this chapter answers: What am I really going to pay?

Who it's for: Anyone navigating this decision.

When you need it: Before you fall in love with a community you can't sustain.

*Note: Dollar amounts in this chapter reflect 2026 rates. All patient stories in this guide are de-identified composites. The information provided in this guide is for general educational purposes only and **DOES NOT CONSTITUTE MEDICAL ADVICE**, psychological advice, or legal advice. Reading or using this material does not establish a physician-patient relationship with Dr. Michael Genualdi or The Geriatrician's Guide LLC. You should always consult with your own qualified healthcare provider or medical professional regarding any specific medical conditions, symptoms, or senior care decisions.*

Nationally in 2026, assisted living runs around \$6,200 a month on average, roughly \$74,000 a year. But the average hides an enormous spread. In lower-cost regions of the country you might find good care well under \$4,500 a month; in major coastal metros, the same level of care can run \$8,000 or more, and luxury communities in premium markets reach \$10,000 to \$13,000. Memory care costs more, averaging around \$7,000 a month and not unusual above \$8,000, because it requires more staff, more security, and more specialized training. A skilled nursing facility, for long-term custodial care rather than short rehab, runs higher still, commonly \$9,000 to \$12,000 a month and up.

The advertised price is almost never what you'll actually pay.

The number on the brochure is the base rate: rent, meals, housekeeping, activities. On top of it sits a second layer most communities don't volunteer on the tour: **care fees**, priced by level. When your loved one needs help with medications, bathing, dressing, or transfers, the community assesses them and assigns a care level, and each level adds to the monthly bill. That care-level assignment is based in part on a physician's assessment

of your loved one's needs, which means the accuracy and completeness of that document has direct financial consequences, not just clinical ones. These add-ons are not small. They can add hundreds to thousands of dollars a month to the base rate, and they rise as your loved one's needs rise, which they will. Budgeting for the base rate and then watching it climb by several thousand a month over eighteen months is not the exception. It's the pattern.

Then there are the one-time costs. Most communities charge a community fee or move-in fee at the start, frequently several thousand dollars, sometimes a full month's rent or more, often non-refundable. Some require a deposit on top of that. Read every one of these before you sign, because they're easy to gloss over in the relief of having found a place.

What I see when I walk in:

I cared for a man whose family had done everything right. They researched communities, toured carefully, chose a place that genuinely suited him: attentive staff, close to his daughter's house, activities and other residents he engaged with. The base rate was a stretch, but manageable. For about a year, it worked.

Then he had a fall. He recovered, but not fully. He needed more help transferring, and his medications required closer monitoring. The community reassessed him and moved him to a higher care level. The monthly bill went up by nearly \$900. Six months later, the addition of new medications prompted another reassessment. Another level, another increase. Within eighteen months of move-in, the family was paying close to \$2,000 more per month than they'd planned for, on top of the community's annual rate increase.

They couldn't sustain it. He had to move. A life that had been working, that he had settled into, that his family had finally stopped worrying about, came apart. Not because the care was wrong, but because the financial picture had been built on the entry price instead of the trajectory. His family found themselves back at the beginning of a search that was now harder in every way: he was frailer, more disoriented by change, and his health had added care requirements that narrowed which communities could even accept him.

This was not the only time I've seen this happen. Far from it. The community didn't do anything wrong. The family didn't do anything wrong. The gap was in the planning, and my patient paid the price.

Two forces push the cost up every year: your loved one's care needs increase as they age, moving them into higher and more expensive care levels, and the community raises its rates annually, commonly a few percent, sometimes more. Stack those together over several years and the monthly cost can climb substantially from where it started. Chapter 18 will help you project this honestly. For now, just hold the principle: budget for the trajectory, not the entry price.

Deep Dive: *All-Inclusive vs. Base-Plus-Care-Level Pricing*

Communities price one of two ways, and the difference compounds over time. An **all-inclusive** model charges one monthly fee that covers care at any level: the price doesn't jump when your loved one needs more help. A **base-plus-care-level** model charges a lower starting rate and adds tiered care fees on top as needs rise.

For a low-needs resident who stays low-needs, base-plus is usually cheaper, and the all-inclusive resident is overpaying for care they don't use. For a resident whose needs climb — the common trajectory — the math often flips: the all-inclusive fee that looked high at move-in becomes the better deal, because it insulates against escalating care charges, while the base-plus resident watches the bill ratchet upward level by level. The trap is that people almost always choose on the entry price, where base-plus looks like the bargain, without pricing the trajectory, where it frequently isn't.

When you ask a community for its fee schedule, ask specifically which model they use and request a sample bill at two or three different care levels. That single exercise will tell you more about your real cost trajectory than anything on the tour.

The financial picture here is complex enough to warrant outside help. If your loved one has a financial advisor, this is exactly the kind of projection they should be running. If they don't, a fee-only fiduciary can help. This is a professional who charges for their time

rather than earning commissions, and who is legally required to act in your loved one's best interest. A couple of hours reviewing your loved one's financial situation with a trained professional is worth far more than discovering the gap after the move-in.

The one thing to do this week:

For any community you're seriously considering, ask for the complete written fee schedule: base rate, all care levels and their triggers, the community fee, the deposit, and the rate-increase history for the last three years. In writing. Then you'll know the real number.

If you take nothing else:

Care fees and annual increases mean the real bill climbs over time. Budget for where this goes, not where it starts. A fiduciary can help you find that number before it finds you.

THE
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The Long Road

A Geriatrician's Complete Guide
to Senior Care Decisions



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READ THE FULL BOOK

This is one chapter of forty-one.

The rest cover the full arc of senior care decisions: the financial mechanics, and how to make a plan that holds; what to look for once you're touring; and how to support and advocate for your loved one. Through decline, through dementia, and the end-of-life decisions almost no one is taught how to make. I hope you'll let me guide you.

— *Dr. Michael Genualdi*

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